

ST. VINCENT CARDIOLOGY CONSULTATION REPORT
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Date: November 02, 2024 | Location: Cardiac Care Unit (CCU)

PATIENT DEMOGRAPHICS

Name: Michael Chang
Age: 58 | Gender: Male | MRN: CCU-882194
Consulting Physician: Dr. Sarah Jenkins, MD (Cardiology)

REASON FOR CONSULTATION

STAT Cardiology evaluation for acute onset substernal chest pressure (8/10 severity) radiating to jaw, accompanied by diaphoresis and dyspnea.

PAST MEDICAL HISTORY

- 1. Type 2 Diabetes Mellitus (15 years)
- 2. Hyperlipidemia
- 3. Coronary Artery Disease (s/p PCI with drug-eluting stent in 2020)

ALLERGIES

Sulfa Drugs (Anaphylaxis documented in 2016)

VITAL SIGNS & CLINICAL EXAM

BP: 168/98 mmHg | HR: 110 bpm | RR: 24/min | SpO2: 93% on room air | Temp: 98.8 F

STAT LABORATORY & DIAGNOSTIC FINDINGS

- Cardiac Troponin I: 0.85 ng/mL (Reference Range: 0.00 - 0.04 ng/mL) [CRITICAL HIGH]
- Glucose (Fasting): 210 mg/dL (Reference Range: 70 - 99 mg/dL) [HIGH]
- B-Type Natriuretic Peptide (BNP): 680 pg/mL (Reference Range: <100 pg/mL) [HIGH]
- ECG: 2mm ST-segment elevation in leads V2-V4 consistent with Acute Anterior STEMI.

DISCHARGE / TRANSFER MEDICATIONS

- 1. Aspirin 325 mg PO immediately
- 2. Heparin IV drip protocol
- 3. Atorvastatin 80 mg PO daily
- 4. Trimethoprim-Sulfamethoxazole (Bactrim) DS 1 tab PO daily (Note: Check allergy profile)

RECOMMENDED NEXT STEPS & CLINICAL PLAN

- 1. Emergency Cardiac Catheterization / Primary PCI immediately.
- 2. Transfer to Cardiac Intensive Care Unit (CICU).
- 3. Contraindication alert: Cancel Bactrim prescription due to severe Sulfa allergy.